A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Dr Jonathan Nettle
Frances Perry House

Dear Dr Nettle,

Re: Mrs Patricia Foley
DOB: 1985-04-26
Address: 41 Malaya Rd, PUCKAPUNYAL VIC 3662

Summary

- **No histological evidence of active Crohn's disease on recent biopsies.**
- **Clinically stable to proceed with IVF and pregnancy.**
- **For post-partum review and surveillance faecal calprotectin.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Patricia Foley to discuss the results of her recent investigations and her future management, particularly in the context of her planned IVF treatment.

As you are aware, there has been a diagnostic question regarding Crohn's disease given her clinical history and previous imaging. I reviewed the results of her recent gastroscopy and biopsies with her. Reassuringly, the biopsies did not demonstrate any histological evidence of active Crohn's disease or significant inflammation. While previous MRI findings and symptoms provided some suspicion of an inflammatory bowel process, the lack of current histological activity suggests two possibilities: either Patricia does not have Crohn's disease, or the condition is currently in a state of remission.

We discussed at length the implications of this for her upcoming IVF and pregnancy. I have advised Patricia that based on her current clinical and histological status, there is no contraindication to proceeding with IVF or pregnancy. The risk of an aggressive acceleration of undiagnosed Crohn's disease in this setting is exceedingly low.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Impression and Plan Patricia does not currently meet the diagnostic criteria for Crohn's disease to warrant the commencement of maintenance therapy. I have reassured her that should she experience a symptomatic flare during pregnancy, budesonide remains a safe and effective therapeutic option.

For now, our plan is to defer further invasive investigations. I have asked Patricia to contact me if she develops any new gastrointestinal symptoms; otherwise, I would like to review her approximately 12 months post-partum. At that stage, we may consider a repeat faecal calprotectin or further surveillance. I have provided this update to her other GP, Dr Deborah McKinley, to ensure continuity of care.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Deborah McKinlay

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